

MEDICAL HOMEBOUND INSTRUCTION

Loudoun County Public Schools (LCPS) offers medical homebound instruction as part of the continuum of services to students who are enrolled in LCPS.

A. Purpose of Medical Homebound Instruction. The purpose of medical homebound instruction is to sustain the continuity of instruction and to facilitate return to the classroom for those eligible students who must be temporarily confined at home or in a health care facility due to their medical needs, both physical and psychiatric, that do not allow school attendance for a limited period of time. Medical Homebound services may be used to supplement the classroom program for students with health impairments whose conditions may interfere with regular school attendance (e.g. students receiving dialysis or radiation/chemotherapy; or students with other serious health conditions). Medical Homebound services are temporary and not intended to supplant school services. A student receiving medical homebound instruction remains enrolled in their attending school and the attending school maintains all aspects of responsibility for the student including but not limited to staffing homebound teachers, grades, attendance, and discipline.

B. Eligibility for Medical Homebound Instruction.

1. Medical homebound instruction shall be considered for students whose parent or guardian has submitted a completed medical certification of need which is fully completed by a medical doctor or nurse practitioner. If the student is referred for a mental health illness, the certifying health provider must be a psychiatrist, psychiatric nurse practitioner, or licensed clinical psychologist. Additionally, parent consent to contact treating provider must be given. School teams must consider all reasonable accommodations for school attendance and alternatives to medical homebound prior to submitting a medical homebound referral. Approval decisions require a minimum of 15 school day predicted absence and are made by the Homebound Coordinator or Specialist. Medical homebound instruction is not appropriate to compensate for absences related to family care or employment. Approval durations may be up to 9 weeks in duration. Extensions are available with updated certifications of need. Medical homebound is designed to support schools during the regular school calendar.

C. Implementation of Services.

1. Each school principal will designate a Medical Homebound Contact at their school. This individual will be a principal or an assistant principal. Secondary student support advisors may also serve in that role under the supervision of a principal or assistant principal. This individual will serve as the primary link between the school and the central homebound office.

2. When it is reported by the parent that a student will be absent for an extended period due to a medical or psychiatric need which cannot be addressed through school-based accommodations, the homebound contact at the student's school will

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provide the parent with a School and Parent Request for Homebound and a Physician's Certification of Need form. These completed forms will be sent to the Homebound office by the medical homebound contact.

3. Following the receipt of the Certification of Need and School and Parent Request, the Medical Homebound Services Coordinator submits notification of the determination decision and the dates of approval to the school-based team and to the Special Education Supervisor and/or 504 Coordinator, if required.

4. Medical homebound instruction should be initiated as soon as possible following receipt of a completed medical certification of need, but no later than five instructional days upon approval of the request. Students who experience administrative delays in implementation are eligible for compensatory services.

5. Medical homebound instruction may be:

- a. provided to LCPS students who are confined in another county or city in Virginia and to qualified students confined in another state who meet medical homebound eligibility requirements,
- b. supplemental, or part-time, for LCPS students with health impairments whose conditions may interfere with regular school attendance,
- c. provided on an intermittent basis for students with a chronic illness or condition, if recommended by the treating physician, nurse practitioner, psychiatrist, psychiatric nurse practitioner, or licensed clinical psychologist and approved by the Homebound Coordinator,
- d. provided part-time in the school setting for students who are unable to participate in full day instruction within the classroom setting,
- e. extended beyond nine weeks with the required physicians request for an extension as submitted to the school homebound contact by the parent.
- f. provided virtually or in an alternative setting if the home is not deemed appropriate for instruction and the student's condition allows.

6. Instructional priority will be provided for core academic subjects (Language Arts, Math, Science and Social Science). Advanced Placement, electives, and special courses will be considered but are not guaranteed.

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If it is determined that the course should be provided, it may not be comparable to the school-based instructional experience. Personal finance, economics, and/or World Language and Culture courses shall be considered for high school students if required for graduation.

7. The following represents the minimum hours of instruction to be provided. These hours may not be applicable in all instances. For students receiving special education, the IEP team will determine the number of hours necessary for students to access homebound instruction:

- a. Elementary school students – one hour per day
- b. Middle school students – eight hours per week
- c. High school students – two hours per core academic subject per week and other accommodations on an individual basis

For students receiving Homebound Instruction who do not have an IEP or Section 504 Plan but are suspected of having a disability, a Child Study team shall meet to determine whether the student should be referred for evaluation for eligibility for an IEP or Section 504 in accordance with Policy [5330](#).

8. Homework should be expected. Not all assignments will be completed in the presence of the homebound teacher. Students should be prepared to participate in the instructional process. This involves attending scheduled sessions, completing assignments, and participating in discussions or activities as required. Where feasible, school-based instructional options should be leveraged to provide a comprehensive educational experience.

9. Students on medical homebound instruction will have access to Standards of Learning Assessments (SOLS) as prescribed by [8VAC 20-131-30](#).

10. Homebound instruction is designed to provide continuity of educational services between the classroom and home or health care facility, for students whose medical needs, physical or psychiatric, do not allow school attendance for a limited period of time. As schools provide a more comprehensive education for students, medical homebound instruction is not intended to be an alternative to school attendance or supplant school instruction. Credit for student work shall be awarded when it is completed under the direction of a licensed teacher and the work meets subject matter content standards and there is evidence that the instructional time requirements or alternative means of awarding credit are in accordance with [8VAC20-131-110](#).

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11. Students participating in medical homebound instruction may be allowed to participate in extra-curricular activities, including sports, if recommended by the treating physician, nurse practitioner, psychiatrist, psychiatric nurse practitioner, or licensed clinical psychologist and approved by the Homebound Coordinator. Students participating in high school sports or other activities under the purview of the Virginia High School League (VHSL) must also be eligible to participate under VHSL rules.

12. Medical homebound instruction shall be terminated after 9 weeks unless a completed reauthorization form including the justification and anticipated timeframe of the extended medical homebound instruction; an updated treatment plan; progress towards treatment goals; and plans to transition the student back to the school setting is provided by the treating health care provider documenting the need for extended instructional support.

13. If the parent or guardian does not agree on the medical homebound services offered by the school, the parent or guardian may submit their concerns in writing to the Coordinator of Homebound Services and request a formal review. The Coordinator for Homebound Services will collaborate with the Director of Student Services and/or the Director of Special Education to review the concerns and communicate the decision to the parent or guardian and school-based contact within 5 business days. The decision rendered will be final.

D. Medical homebound instruction for students receiving special education.

1. Homebound services shall be subject to review by the student's Individualized Education Program (IEP) team pursuant to the Individuals with Disabilities Education Act. As part of the review the IEP Team must:

- a. review the homebound courses recommended by the Homebound Coordinator and/or school-based team,
- b. review the approved Medical Certification of Need,
- c. provide parents with Prior Written Notice and obtain informed parental consent to amend the students IEP prior to initiation of medical homebound instruction,
- d. determine the appropriate placement for the student based on the student's educational needs,
- e. revise the IEP and include language in the IEP that clearly determines if special education instructional services are required, and if so, the method of instruction, where medical homebound instruction will be provided, the time period, frequency and duration of services, and the numb of hours of instruction per week,

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- f. include a statement that stipulates that medical homebound instruction is temporary and does not constitute a permanent change in placement and is not considered a “stay put” placement,
- g. amend the IEP prior to the conclusion of services, and either return the student to the school setting, continue medical homebound instruction or place the student in an alternative placement as determined by the IEP team.

E. Medical homebound instruction for students receiving 504 services.

1. Medical homebound instruction for students receiving 504 services may be reviewed by the 504 team to determine if accommodations are required during homebound instruction. The 504 Plan may be reviewed by the team as appropriate.

F. Teachers Qualifications. Homebound teachers should have a broad background of professional training and experience to adapt instruction to each student’s needs. In determining the appropriate teacher to provide [homebound] instruction the same requirements that must be met for “in school” instruction must be met for those students’ receiving instruction at home. Matching teacher credentials/qualifications to student needs will need to be done on a case-by-case basis. Teachers eligible to provide medical homebound instruction must hold an active Virginia license issued in accordance with rules and regulations of the Virginia Board of Education. Teachers may provide instruction outside of their licensure endorsement when supervised by a highly qualified teacher.

[Former 5-64]

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Leg Refs.: 8VAC20-131-180, 8VAC20-131-110, 8VAC20-131-30

Cross Refs.: Policy 5330 Special Education Student Eligibility and Transfer Procedures, Policy 8140, Student Attendance Requirements and Procedures

Virginia Department of Education Homebound Instructional Services Guidelines
Regulations Governing Special Education Programs for Children with Disabilities in Virginia